

PRIORITIZATION

NCLEX STUDY GUIDE

ABCD • Triage • Delegation • Emergency Management

1. Core Priority Framework

- Maslow: physiologic needs generally precede psychosocial needs.
- ABCD: Airway, Breathing, Circulation, Disability/neurologic status.
- Assess the client before equipment whenever possible; treat the most immediate threat to life.
- Pain is not usually first unless it signals a life-threatening problem such as chest pain, epigastric pain in pregnancy, DKA, pancreatitis or Addisonian crisis.

FIRST LOOK Unstable, unexpected, acute and newly changed findings take priority over stable, expected or chronic findings.

2. Airway Priorities

Condition	Priority cues / action
Stridor	Priority airway sign. Epiglottitis: drooling, avoid oral examination, keep emergency airway equipment available.
Anaphylaxis	Hives may be an early cue; rapidly assess airway and circulation.
Facial/inhalation burn	Singed nasal hair/eyelashes, soot or carbonaceous sputum, cough and enclosed-space exposure.
Post-thyroidectomy	Bleeding behind neck, dyspnea, laryngeal spasm or tetany; emergency airway set at bedside.
Dislodged tracheostomy	First 72 hr: ventilate with bag-mask and activate emergency response. Mature tract: use obturator/dilator per policy and call for help.
Foreign-body aspiration	Muffled “hot-potato” voice, stridor, inability to speak in sentences.
Superior vena cava syndrome	Periorbital edema, tight-collar sensation, upper-extremity edema, dyspnea/stridor; oncologic emergency.
Newborn distress	Grunting, nasal flaring, seesaw breathing and intercostal/subcostal retractions.

3. Breathing Priorities

Problem	Critical finding
Respiratory depression	Opioids, magnesium sulfate, phenobarbital, lidocaine or benzodiazepines; RR below 8 is an emergency.
Cervical spinal injury	C3-C5 affect the diaphragm; C4 compromise is especially critical.
Neuromuscular weakness	Guillain-Barré, myasthenia gravis and botulism become priorities with dyspnea or respiratory-muscle weakness.
Chest trauma	Pneumothorax: unilateral absent sounds/crepitus. Tension: contralateral tracheal deviation. Open: sucking wound. Flail chest: paradoxical motion.
Pulmonary edema	Pink frothy sputum and bilateral ascending crackles.
Status asthmaticus	“Silent chest,” worsening dyspnea and pulsus paradoxus.
Pulmonary embolism	Sudden dyspnea, tachypnea, cyanosis, chest pain, hemoptysis and tachycardia; consider DVT, long-bone fracture or central-line history.

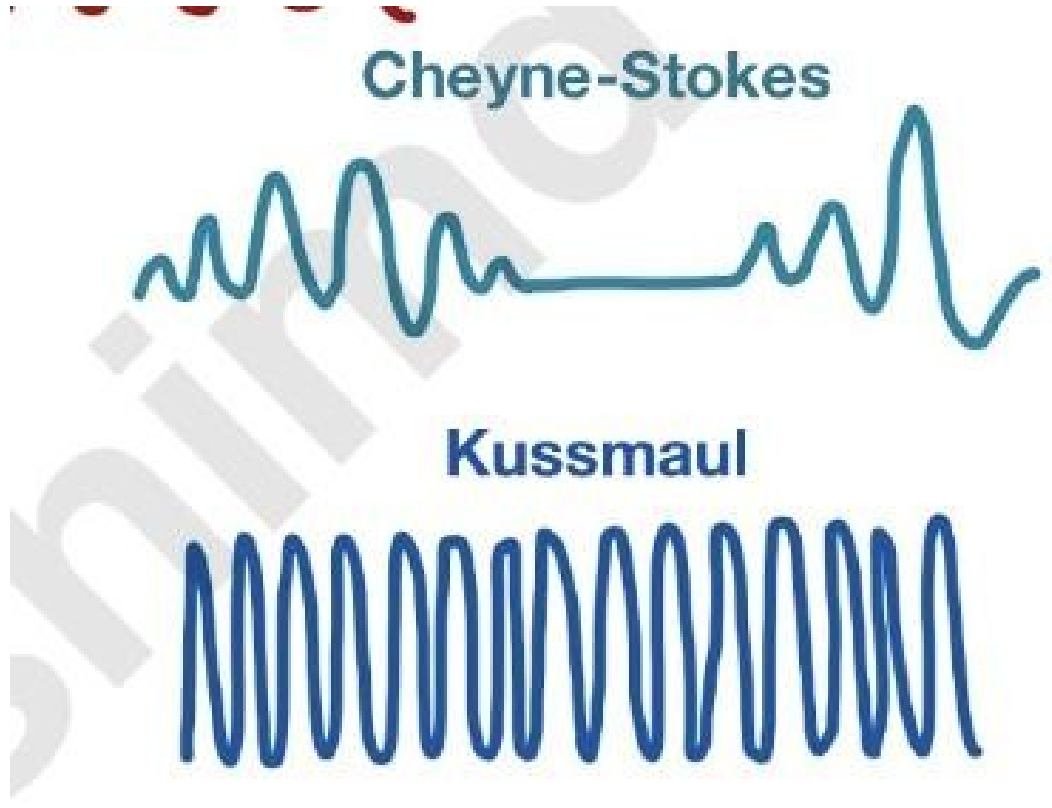
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Chest tube chamber	Priority finding
Drainage collection	Bright-red drainage or unexpectedly high output after the initial period.
Water seal	Continuous bubbling suggests an air leak; absent tidaling may mean re-expansion, kink or obstruction.
Suction control	Assess system according to device type and prescribed suction.

VENTILATOR ALARMS High-pressure alarm usually points to the patient/obstruction. Low-pressure alarm usually points to disconnection or machine leak. Assess the patient first and provide bag ventilation if needed.

4. Circulation Priorities

- Chest or epigastric pain may signal myocardial infarction; in pregnancy, epigastric pain may signal severe preeclampsia.
- Addisonian crisis: hypotension, tachycardia, abdominal pain, hyperkalemia and shock.
- DKA: fruity breath and Kussmaul respirations; metabolic acidosis is immediately treatable.
- Shock: hypotension plus tachycardia. Prioritize hemorrhage, ruptured ectopic pregnancy, severe dehydration and active blood loss.



Cheyne-Stokes and Kussmaul respiratory patterns

Rhythm/problem	Priority response
PVCs	Concerning when multifocal, frequent, paired or in runs after MI.
Ventricular tachycardia	Check pulse. Stable with pulse: medication; unstable with pulse: synchronized cardioversion; pulseless: CPR and defibrillation.
Ventricular fibrillation	No cardiac output: immediate CPR/defibrillation.
Torsades de pointes	Prolonged QT; evaluate magnesium/calcium and medications.
Potassium changes	Hyperkalemia: peaked T waves. Hypokalemia: U waves.
Digoxin toxicity	Early nausea/vomiting/diarrhea and yellow-green visual halos.
Thyroid storm	Fever plus tachycardia; propranolol and antithyroid treatment as ordered.

Other circulation emergencies

- DIC: diffuse oozing from IV sites; associated with obstetric emergencies and severe illness.
- Compartment syndrome: severe pain not relieved by medication and diminished pulses after cast, burn or snakebite.
- Testicular torsion: sudden severe pain and absent cremasteric reflex.
- Tumor lysis syndrome: low calcium with elevated potassium/phosphate/uric acid after chemotherapy; hydrate and give allopurinol as ordered.
- Cardiac tamponade: muffled heart sounds, JVD with clear lungs and pulsus paradoxus.
- Tracheostomy pulsating with heartbeat suggests innominate-artery fistula and catastrophic bleeding risk.

5. Maternity and Neurologic Priorities

Finding	Priority meaning/action
Preeclampsia/eclampsia	Hypertension, proteinuria/edema; seizure indicates eclampsia.
Late deceleration	Uteroplacental insufficiency: stop oxytocin, left lateral position, oxygen as ordered, notify provider.
Variable deceleration	Cord compression; prolonged severe deceleration is an emergency.
Uterine rupture/inversion	Loss of contractions and hemorrhage; immediate emergency response.
Placental abruption	Painful dark-red bleeding; emergency.

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Neurologic problem	Critical cue
Autonomic dysreflexia	Spinal injury above T6 with severe hypertension and bradycardia.
Neurogenic shock	Hypotension with bradycardia.
Cushing triad	Systolic hypertension, bradycardia, widened pulse pressure and altered respirations.
Stroke	Thrombotic: TIA/atherosclerosis history. Embolic: AF/mechanical valve. Hemorrhagic: “worst headache,” cocaine or severe HTN.
Myxedema coma	Severe hypothyroidism with neurologic and respiratory decline.
HHS	Type 2 diabetes with profound hyperglycemia, often above 600 mg/dL, dehydration and neurologic changes.

6. Emergency Department Triage

Code	Time/meaning	Examples
Red	Immediate/emergency	ABCD threat, seizure, chemical eye splash, gunshot wound, pneumothorax, flail chest, tamponade, amputation.
Yellow	Urgent	Open/complex fracture, dislocation, significant wound, fever, surgical abdomen, asthma without severe distress, renal stone.
Green	Nonurgent / walking wounded	Closed fracture, contusion, abrasion, minor laceration, sprain, cold symptoms, stable anxiety.
Black	Expectant/deceased in mass casualty	Unsurvivable injury, absent brainstem signs or catastrophic burns when resources are limited.

ETHICAL BASIS Disaster triage uses utilitarian reasoning: use limited resources to achieve the greatest overall benefit.

7. Prevention Levels

Primary	Secondary	Tertiary
Prevent initial disease: vaccination, health promotion, education, sunscreen, hearing protection, body mechanics and prenatal folate.	Early detection/screening: mammography, colorectal screening, breast/testicular examination, blood pressure, glucose, dental, skin and BMI screening.	Reduce disability/complications: rehabilitation, PT/OT/speech therapy, post-stroke care and follow-up of diabetes or chronic disease.

8. Delegation

RN	LPN/LVN	UAP
Initial/unstable assessment; admission, teaching, care plan, discharge; IV push/high-risk IV drugs, blood products, chemotherapy, central lines, legal issues and aspiration-risk feeding.	Stable/chronic clients; oral/IM/SQ/topical medication; tube feeding/medication, Foley insertion, tracheostomy and wound/ostomy care; reinforce teaching and collect ongoing focused data.	ADLs, hygiene, stable vital signs, I&O, ambulation, feeding without aspiration risk, postmortem care, specimen transport, bladder scan, compression devices and routine glucose checks.

DELEGATION RULE The RN retains assessment, clinical judgment, initial teaching and evaluation. Delegate predictable tasks for stable clients, then supervise and follow up.

9. Floating Nurses

- Do not assign a float nurse new admissions, discharge teaching or unfamiliar unstable care. Give stable clients whose needs match the nurse's home-unit skills.
- ICU/ED nurses can usually accept higher-acuity monitored clients.
- OB nurses may take stable postoperative wound care and conditions involving familiar skills such as magnesium therapy or epidural care.
- Pediatric nurses may accept familiar diagnoses such as cystic fibrosis, sickle-cell disease, asthma or appendicitis.
- PACU nurses are familiar with early postoperative hemorrhage, dehydration, electrolyte imbalance and malignant hyperthermia.

10. Emergency Management and Fire Safety

Mitigation	Preparedness	Response	Recovery
Prevent or reduce future harm; insurance and hazard reduction.	Plans, drills, evacuation routes, food/water and supplies before an emergency.	Immediate safe action during the event; shelter, evacuate and shut utilities when directed.	Return to normal or a safer condition after the event.

one R.A.C.E you have to P.A.S.S.



R = RESCUE humans in immediate danger , should not endanger your own life
A = ALARM: sound the alarm by activating a pull station alarm box and call 112
C = CONFINE the fire by closing all doors and windows
E = EXTINGUISH the fire with a fire extinguisher.
EVACUATE the area if the fire is too large for a fire extinguisher.

P = PULL the pin on the fire extinguisher
A = AIM the extinguisher nozzle at the base of the fire
S = SQUEEZE or press the handle
S = SWEEP from side to side until the fire appears to be out



THE R.A.C.E. ACRONYM AND P.A.S.S.FIRE EXTINGUISHER USE

Fire response: RACE and fire-extinguisher use: PASS

- RACE: Rescue, Alarm, Confine, Extinguish/Evacuate.
- PASS: Pull, Aim, Squeeze, Sweep.
- Evacuate clients in immediate danger first, then ambulatory clients, wheelchair/crib clients, and finally bedbound clients. Move clients who cannot be evacuated away from windows and protect them according to the emergency plan.

FINAL NCLEX FOCUS Prioritize actual or imminent loss of airway, ventilation, perfusion or neurologic function. Choose unstable and unexpected findings before stable chronic findings, then use scope-of-practice rules for delegation.